

Subject: Fall prevention

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From: Barnett, Michael

To: Boynton, Harrison, Griffin, Rebekah, White, Brooke, Zamora, Joel

I like the idea of focusing our first protocol for the virtual house on fall prevention. It could be interesting to study among older adults with and without cognitive impairment but also with their caregivers. It could ultimately even be something home health type places might want to use to train staff.

Brooke, I think it would be good to pull resources on common risks for falls — environmental risks, I mean. I mean, we came up with a bunch, but there are probably other things we're not thinking of. But slippery floors, loose rugs, broken handrails, clutter, cords/wires, etc.

We'd need to describe it carefully. It's not a measure of fall proneness per se — it's almost metacognition about risk of falls in a given environment. It's going to be one of these classic scenarios in which we always find ourselves: it's going to be so obvious that it measures something important and yet very hard to validate since we're measuring such a basic skill. Could we ask an RA to start pulling articles? We want to see if something like this already exists with or without VR. I know there are little questionnaires/checklists out there. We're going to need some stuff like that to get it. We may want to reach out to OT folks to see if they have anything they use.

I love it when random ideas like that pop up. Harrison mentioned the NAB subtest, I had a weird incident in which I nearly fell on a staircase at a concert hall yesterday, and Brooke was talking about the house, and the idea occurred! Falls are the number one reason people wind up in SNFs. One nice thing would be that we could apply for grants dealing with Parkinson's, DLB, etc. and not just AD.

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